Submitted via email to*: https://www.regulations.gov*

June 16, 2025

Mehmet Oz, M.D., Administrator, Centers for Medicare & Medicaid Services
Thomas Keane, M.D., Assistant Secretary for Technology Policy and National Coordinator for Health Information Technology

Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS–0042–NC
P.O. Box 8013
Baltimore, MD 21244–8013

RE: Request for Information on Health Technology Ecosystem [CMS–0042–NC]

Dear Dr. Oz and Dr. Keane:

The Healthcare Trust Institute appreciates the opportunity to submit comments in response to the Request for Information issued by the Department of Health and Human Services' (HHS) Centers for Medicare & Medicaid Services (CMS) and the Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC).[1]

The Healthcare Trust Institute (HTI) is an alliance of healthcare organizations committed to promoting and implementing effective privacy and security protections for health information that engender trust in the healthcare system and allow for the advancement of treatments, cures and improved healthcare quality for individuals and populations. HTI members, which include companies and organizations from across the U.S. healthcare economy, support the existing Health Information Portability and Accountability Act (HIPAA) framework that has long provided robust privacy and security protections for a

[1] 90 Fed. Reg. at 21034 (May 16, 2025).

patient's protected health information (PHI). We also believe that a strong national privacy standard is needed to protect personal health information outside the HIPAA framework.

We strongly support the goals of CMS and ASTP/ONC to build a patient-centric health ecosystem through the increased adoption of digital health products to improve health data access and care, resulting in better health outcomes. We also agree that while HHS has established a policy framework to promote the seamless and secure flow of health information among patients, providers, and payers, there is still more that can be done to improve and simplify the process for all stakeholders, and that the greater availability and use of digital health products will reduce paperwork and telephone calls.

A foundational prerequisite for a streamlined and patient-centric digital health ecosystem is passage of a comprehensive federal law to protect consumer personal information that falls outside of HIPAA. This also critical for the development and adoption of artificial intelligence (AI) tools in the health sector, given the centrality of data to AI. Without the assurance that their personal data will remain protected and used and disclosed appropriately, patients and consumers will lack the trust essential for their acceptance of AI solutions and digital health products. This law should incorporate the key tenets of the HIPAA framework, which have proven to be most effective in protecting patient privacy and engendering trust, and should harmonize with HIPAA in its approach, concepts, and definitions. This federal law and HIPAA should indisputably preempt state laws dealing with health data privacy, rather than simply setting a federal floor as HIPAA currently does.

The need for broad federal preemption has become particularly urgent as more states have stepped in to pass comprehensive data protection laws in the absence of progress at the federal level,[2] creating a thicket of different data protection laws. This is not only confusing and difficult for patients and consumers to navigate, but also burdensome and costly for businesses to comply. It also inhibits technological improvements and innovation, particularly around AI, as different rules for the use of data needed to develop and deploy AI solutions apply in different states, imposing operational and compliance barriers.

Below we provide recommendations to accelerate the adoption of health information technology to improve health data exchange and interoperability, promote innovation in and utilization of digital health products, and advance the transition to a value-based care (VBC) model.

[2] To date, 20 states have adopted generally applicable privacy laws, and this number is expected to grow as long as Congress does not pass a national privacy law that applies to personal data, including personal health data not already subject to HIPAA. See https://iapp.org/resources/article/us-state-privacy-legislation-tracker/.

1. Digital Health Product Certification

The RFI asks what CMS, and its partners can do to encourage patient and caregiver interest in digital health products and, specifically, what role CMS should have in reviewing or approving digital health products on the basis of their efficacy, quality, and/or impact on health outcomes (PC–5).

A key concern for patients is that their health information will be protected and only used and disclosed in accordance with the patient's reasonable expectations. HHS can play a critical role in encouraging consumers to use digital health products by assuring them that the health app developer to which they entrust their health information has been vetted and determined to meet appropriate privacy and security standards. This can be done by HHS developing a certification program by which health apps can be evaluated for this purpose and issued a certification if they meet the specified privacy and security standards. Certifications can be performed by independent and qualified organizations approved to do so. This is especially important today, given that many of these digital health products are not subject to a national privacy and security law. But even if and when such a law is passed, this type of certification program can help patients choose digital health products that they have the assurance meets high privacy and security standards. Without this assurance, consumers are less likely to adopt digital health products on the scale needed to transform the health ecosystem as envisioned by CMS and ASTP/ONC.

2. Digital Identity Credentials

The RFI asks several questions regarding the use of digital identity credentials, including the benefits and downsides, the challenges to adoption and the potential role of Credential Service Providers (CSPs) to improve access to health information, reduce patient and provider burden, and enhance identity management and security. (See PC-13 and 14, PR-9 to 11, PA-3 and 4 and TD-3)

We support the use of technologies that can improve the verification process by making it more secure and less burdensome. We believe that entities such as CLEAR, Login.gov, ID.me, and other NIST 800–63–3 IAL2/AAL2 CSPs can play an important role in this regard, giving providers and payers greater confidence that an individual is who they profess to be, and allowing for a simpler process for patients and caregivers to verify their identity. We note, however, that there may also be other technologies or mechanisms for identity verification, such as passkeys and digital drivers' licenses, each with their own benefits and downsides. We recommend that all these technologies, and potentially others, be considered for verification, provided they are secure and reliable, rather than settling on only one technology. This will ensure ongoing improvement and innovation in identify verification technologies.

While we agree that CSPs may improve access to health information, we caution that in the healthcare context, identity verification is only one component of a more complex process that also involves ensuring that the correct health records are accessed (i.e., patient matching) and that access by the CSP is authorized by the patient where required by law. Generally, the responsibility for all these components lies with the HIPAA covered entity or its business associate providing access to the patient's health information. This then raises the question as to the role and accountability of CSPs that are chosen by patients and must be accepted and relied on by HIPAA entities to allow access to the patient's record. For example, if the CSP chosen by a patient makes an error that results in a healthcare provider giving access to a patient's records to the wrong person, the CSP should clearly be responsible. This is not only a matter of fairness, but also to ensure that CSPs have appropriate incentives to ensure that they protect patient data from falling into the hands of unauthorized parties. CMS and ASTP/ONC should consider what changes, in statute or regulation, would be needed to ensure that patients do not bear the burden of having to hold CSPs responsible contractually for any failures in the performance of their responsibilities with respect to the patient's health data. Ideally, health apps and CSPs would be covered by a national privacy law, but until such a law is passed, HHS could by regulation deem CSPs to be business associates of the HIPAA entity whose records they access for the patient. In either case, CSPs would bear responsibility under a federal privacy law for accurately verifying the identity of the patient whose records are accessed, providing a critical protection for patients and the HIPAA entities whose records are accessed.

3. TEFCA

The RFI asks whether the Trusted Exchange Framework and Common Agreement (TEFCA) is currently helping to advance patient and provider access to health information, and if there are adequate alternatives available outside of TEFCA (PC–10 and PR–6). The RFI also asks about the policy or technical limitations of TEFCA, and whether these hinder participation in TEFCA (PA–1), and finally, what unique interoperability functions TEFCA performs.

TEFCA currently performs a unique role in promoting interoperability by creating a national framework and standards for health data sharing and connecting health information networks (HINs) across the country. While we continue to support the voluntary nature of TEFCA participation, and caution against expanding to new use cases unless fundamentals such as patient privacy are in place, we believe there are several steps HHS can expand participation in TEFCA. This could involve education, incentives, and other forms of engagement, such as through the provision of technical and financial assistance to providers that may have limited technical capabilities and resources. It is also important that fees for participation are appropriate, non-duplicative, and scaled to the size and potential use by participants.

TEFCA participation may also be encouraged by increasing the utility of TEFCA participation to patients and consumers. One feature that would benefit patients is to incorporate a mechanism for patients to correct or add to their records. While existing methods allow patients to upload individual files to specific providers, these uploads do not become part of the patient's interoperable record. In addition, the TEFCA Individual Access Service (IAS) request is limited to access and deletion options. It would be beneficial if patients were also provided an option to amend or augment their records through a patient portal so that these changes could be automatically incorporated into their records exchanged through TEFCA. Another way to encourage participation in TEFCA is by improving the functionality of existing technology and services provided by QHINs. For example, requiring that all QHINs meet certain standards for patient matching and record location technology would improve data exchange and help ensure that none of a patient's records are unintentionally missed or omitted.

CMS could also promote TEFCA's role as a national health data sharing framework by making its own data, including not only its encounter and claims data, but also its research and public health data, available through TEFCA, with CMS or a division of CMS potentially becoming a QHIN. The richness of the CMS databases is unsurpassed, and the availability of data from these, consistent with all applicable privacy and security requirements, would be a major boost to TEFCA and cement its role as the pre-eminent framework for health data exchange in the United States.

The value of CMS participation in trusted exchange lies not only in the breadth and depth of its databases, but also because it would be a trusted, cost-effective option alternative for patients, providers, and others. We anticipate that CMS' role will be primarily as a provider of EHI, rather than a requestor since CMS has other more appropriate mechanisms and authorities by which to collect data its needs for oversight and similar purposes.

Since data is at the heart of interoperability, for TEFCA to be successful it is critical that data privacy and security protections are robust throughout the data exchange process. This applies to all health data, whether held by HIPAA entities or those operating outside HIPAA. it is also important that TEFCA participants and those that exchange EHI with them have the assurance that TEFCA requirements are properly enforced, and that prompt action is taken against entities that violate the TEFCA requirements. ASTP/ONC should consider whether additional steps are needed to protect TEFCA participants from bad actors that misrepresent the purpose for which they seek EHI. This has been an ongoing concern under TEFCA,[3] particularly as the HITECH Act's requirement to share patient information with a third party at the patient's direction at minimal cost encourages some

[3] See "How to Stop Abuses in Patient Health Exchange" by Tina Grande and Deven McGraw (Health Affairs, June 18, 2024).

entities seeking data for other purposes to misrepresent that they are acting on behalf of the patient.[4]

While we understand that QHINs have the primary responsibility for ensuring that their participants abide by the TEFCA requirements and purposes, this responsibility may be more than QHINs and even the recognized coordinating entity (RCE) can handle on their own, particularly if TEFCA expands as hoped. As the regulatory agency invested by Congress with the authority and responsibility for promoting national interoperability, ASTP/ONC should consider investing additional resources to monitor and enforce compliance with the TEFCA requirements, making the case to Congress as needed to increase the funding available for this purpose. In addition, as discussed further below, it should review the information blocking rule and exceptions to ensure that TEFCA participants and other actors are protected from entities that are not acting in good faith and seeking to exploit TEFCA and HIPAA for their own gain and at the expense of individuals' privacy while ensuring that those with a legitimate basis for seeking data do not face unnecessary hurdles.

Striking this balance between privacy protection and appropriate access is critical, and once an actor has confirmed that a request is for a valid purpose, it should be required to provide access without imposing hurdles that are inconsistent with the intent of the Cures Act. ASTP/ONC and CMS should use existing authorities to investigate complaints of violations and make determinations about an organization's ongoing ability to participate in a trusted exchange.

Finally, It is also important that the process for the adoption of TEFCA requirements be transparent, well understood, and allow for stakeholder input. This is particularly the case in light of the fact that certain practices related to TEFCA requirements that involve withholding or restricting EHI do not constitute information blocking. To ensure that such TEFCA requirements are appropriately vetted, they should also be codified in regulation. All substantive issues and core concepts, such as, but not limited to, foundational definitions of the different exchange purposes, should be codified in regulation following the notice and comment rulemaking process, rather than being addressed only in TEFCA documents such as SOPs. This is essential to ensure that that TEFCA participants and those interacting with them trust and have full confidence in the TEFCA framework. It is only when all entities in the healthcare sector engaged in the exchange of EHI understand, trust, and have an opportunity to provide input on the TEFCA framework that it will be embraced and widely adopted, leading to true national interoperability. Consistent with this, we recommend that until passage of a national privacy law that

[4] See Section 13405(e) of the HITECH Act. See also Senator Bill Cassidy's February 2024 Report "Strengthening Health Data Privacy for Americans: Addressing the Challenges of the Modern Era," p.4 ("In some cases, fraudulent and abusive actors take advantage of pathways for requesting medical records by duping patients into signing lengthy forms that allow them to masquerade as the patient, promising payouts through medical malpractice suits.")

applies to non-HIPAA TEFCA participants, any privacy and security requirements for TEFCA participants harmonize with HIPAA unless there are strong policy reasons for applying different standards.

4. Information Blocking

The RFI asks whether ASTP/ONC should consider removing or revising any of the information blocking exceptions or conditions within the exceptions to further the access, exchange, and use of electronic health information (EHI) and to promote market competition (PR–12). It also asks how it may encourage patients, providers, and payers to submit information blocking complaints and the impact of doing so (PC-13, PR-14, and PA-7).

We appreciate ASTP/ONC's continuing efforts to refine the information blocking regulation to promote interoperability and increase the exchange of health information to improve care and health outcomes. We are concerned, however, that as the regulation has evolved, it has become increasingly complex, with overlapping and inter-related exceptions and sub-exceptions and multiple detailed conditions. This is making the regulation increasingly difficult to understand and navigate, despite ASTP/ONC's best efforts to provide educational materials and opportunities. This complexity not only makes compliance more challenging for actors, but also makes it more difficult for requestors to rely on the information blocking regulations to access EHI, as they have to navigate the complex set of exceptions to obtain the health data they need. We therefore encourage ASTP/ONC to consider ways to simplify and streamline the regulation wherever possible, such as through clearer and more precise definitions and simpler exceptions. ASTP/ONC should also endeavor to align the information blocking regulation more closely with other HHS regulations applicable to most stakeholders, such as the HIPAA Privacy and Security Rules. Divergence from these rules creates confusion and unnecessary complexity.

Information blocking rules should support the goal of building longitudinal patient records, which are critical to VBC and other quality-based metrics. Information blocking incentives and disincentives should include creating an environment where data is available for analytics to provide the insights needed to produce better patient outcomes. HTI encourages CMS and ASTP/ONC to examine whether the information blocking rules, and associated penalties are resulting in the type of data sharing and interoperability between providers and other healthcare actors envisioned by the Cures Act, or whether it has in some cases created perverse incentives, resulting in the siloing of large volumes of data to the detriment of patient care.

As the nature of EHI has and continues to evolve, this has added to the complexity, making it more difficult and requiring more resources to adjudicate information requests. As the amount, types and sources of health data that can be accessed or integrated into

information systems increases, further clarification of the definition of EHI would improve information exchange across healthcare entities.
Simplifying the requirements does not necessarily mean broader exceptions. We recommend that ASTP/ONC continue to focus on reducing the overlap and interdependence between information blocking exceptions and sub-exceptions, and work with stakeholders to advance an actionable and workable definition of "interference." We do not recommend codifying certain examples of practices that constitute interference, since this does not in itself provide additional definitional clarity or certainty, as it addresses only the examples listed. In addition, codifying by example could have unintended consequences, including placing undue focus on these practices at the expense of others that are potentially as or more problematic and effectively creating a presumption that the listed practices are information blocking. Exceptions should be narrowly tailored to specific allowable cases that are well-defined and less open to interpretation. With the expectation that information blocking exceptions are just that – exceptions – it becomes imperative that ASTP/ONC prioritize clarity and simplicity so that actors are at less risk of violating the requirements due to misinterpretation or misunderstanding.

Regarding specific exceptions, we recommend that ASTP/ONC consider revising or removing the Protecting Care Access exception and the Individual Request not to Share EHI sub-exception under the Privacy Exception. We are concerned that the threshold condition for the Protecting Care Access exception, namely, the good faith belief that the person is "at risk of potentially being exposed" to legal action is too low, especially when EHI is requested for treatment purposes. The mere disclosure of EHI related to reproductive healthcare is likely to be sufficient to meet this standard, with the result that actors could choose to withhold such EHI at any time without any information blocking repercussions.

Even more concerning is that this could become a slippery slope as there are any number of reasons providers or patients could be concerned about the sharing of certain EHI, whether it is legal action, stigmatization, or some other reason, even when the EHI is not requested for that purpose. We are also concerned with the public policy implications of setting a precedent to vouchsafe protection for actors under the information blocking regulation even in circumstances where those actions are directly contrary to the requirements of another law.

A better balancing of the legitimate fears of patients and providers and the need for EHI for basic healthcare functions is to require a higher threshold, such as that there be an objective basis to indicate that the EHI will be used for legal action against the patient or provider. Alternately, ASTP/ONC could limit the exception to those situations where disclosure would be prohibited if the actor were a HIPAA entity. This would ensure consistency with the HIPAA Privacy Rule and the approach that ASTP/ONC has taken with respect to certain other exceptions (or exclusions from exceptions) so that HIPAA and non-HIPAA entities are treated equally.

We have similar concerns with the Individual Request not to Share EHI sub-exception under the Privacy exception. As with the Protecting Care Access exception, we are concerned about the potential unintended consequences of extending protection under the information blocking rule to actions that an actor chooses to take that are contrary to another law. We believe a better balancing of the privacy concerns of patients and the public good would be to reinstate the limitation that allows the use of this sub-exception unless disclosure is required by law. Aligning the exception with HIPAA reduces complexity and the broader exception may adversely affect patient care (e.g., emergency treatment) if an actor chooses to withhold EHI based on the exception.

We also ask that ASTP/ONC consider making the following revisions or clarifications:

- In the Manner Exception, whether the carve-outs from the fee and licensing requirements in 45 CFR 171.301(a)(2) apply when the parties do not discuss the manner in which EHI is provided, and the requestor simply accepts the EHI in the manner provided. This is a grey area, resulting in inconsistent interpretation.
- In the Licensing Exception, allow the 10-day time frame in 45 CFR 171.303(a)(1) to start once the actor has sufficient information to determine whether the request is appropriate or for a permissible purpose. It is not unusual for there to be some back and forth between the actor and requestor to determine this and, depending on how quickly the requestor responds, the actor or its business associate may need to start licensing negotiations even though it has not yet determined whether it is a request that can be fulfilled.
- Increase the time frame in 45 CFR 171.303(a)(2) from 30 days to at least 90 days or longer, as long as the actor can show it acted in good faith. Most licenses in the health technology industry for interoperability elements that are proprietary to the actor can take anywhere from a few weeks to many months, especially if the requests are for bespoke and/or novel interoperability elements. If negotiations break down after thirty (30) days for legitimate reasons or the requestor delays in responding, the actor should not lose the ability to leverage this exception as long as it can show that it negotiated in good faith.

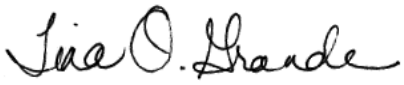
CMS and ASTP/ONC must also make it easier for patients, providers, and digital health developers to submit information blocking complaints, and to turn those complaints into meaningful enforcement action. Many small but cumulative technical barriers or anti-competitive practices may rise to the level of information blocking. By implementing a meaningful process for investigating and acting on information blocking complaints, CMS can ensure continued innovation, giving patients and providers choice in the technologies they leverage to drive better outcomes and lower costs for patients.

Finally, while it is important that there be a mechanism to submit complaints about information blocking and meaningful enforcement for the reasons stated above, ultimately, we believe that the most effective way to promote appropriate health information exchange is through incentives to improve interoperability and clearer and

simpler information blocking rules. A combination of these, alongside the aforementioned enhanced privacy and security protections, should increase information sharing, resulting in fewer disputes and fewer complaints. It will also foster a more collaborative, trusted approach between government and all stakeholders, which is essential for the vision of a truly interoperable health sector to come to fruition.

Thank you for your consideration of our comments. Please do not hesitate to contact me at tina@hctrustinst.com or 202-750-1989 if you have any questions.

Sincerely,



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